Beyond Medicine: Why Global Health Inequality Persists in a Divided World”

The issue of global inequities in healthcare continues to be firmly rooted despite significant growth in advances in medicine, technology, and policies. Despite advances in all the aforementioned fields, inequities in health outcomes across both developed and developing countries continue influenced by the intersection of social class, politics, and inequality (Marmot, 2015). However, technological advances by themselves can never address structural inequities in opportunities created by advances. This essay argues that inequities in global healthcare continue owing to structural violence, inequities of human capabilities, or the social determinants of healthcare proposed by the World Health Organization (WHO, 2008). The examples of both USA and Nigeria will prove how the socioeconomic structure of both countries, among others, continues to frame an individual’s capacity to enjoy healthy lives.

Global health is not merely an issue of medical capacity, but rather an issue of how societies allocate power. As Kickbusch (2014) , "The outcomes of public health measure the practice of social justice." The application of Sen’s (1999) "capabilities" theory reshapes the concept of global health from focusing on medical capacity to focusing on the extension of human freedom. Societies that restrict education, food, or income disparities will inherently restrict someone’s right to healthy choices. Conversely, the World Health Organization’s Conceptual Framework on the Social Determinants of Health states that it is the "conditions of birth, growth, life, or working" which determine the outcome of global health (Irwin et al., 2010). Structural inequality in each of those factors perpetuates cycles of inequity that can never be rectified by advances in medical care. Paul Farmer (2005) represents "structural violence" as systemic lack built into economic, political, or social structures which inherently make populations more susceptible to ill health and premature death.

The US is a manifestation of health inequities amidst prosperity. The US is among the wealthiest countries globally, yet it spends only 17% of its GDP on healthcare. Surprisingly, it trails most other affluent countries on this measure (KFF, 2023). There is a significant difference in life expectancy among various races and socioeconomic classes. The average life expectancy of African Americans is six years less than that of Caucasians. Meanwhile, communities with less income experience doubled chronic diseases such as diabetes and hypertension (Williams & Cooper, 2019; CDC, 2024). Paul Farmer’s (2005) theory states that the ultimate manifestation of inequality is observable in the structural violence imposed on society by race and economic inequities. US population’s poor health outcomes merely depict this fundamental predisposition. Even with optimal medical care available, it is tied to employment, location, or socioeconomic position. Capabilities theory by Sen (1999) articulates how millions of US citizens lack the real freedom of reaching optimal wellness. It combines issues of poverty, lack of education, or lack of accessible care.

The state role of privatization and policy plays an intrinsic role in perpetuating inequities. The healthcare system in the US has a profit-driven focus which allows technological superiority but leaves those who cannot afford it (Navarro, 2021). Marmot (2020) indicated, “causes of the causes” of ill health in the US correlate with “the concentration of wealth” and the destruction of public welfare institutions. It becomes clear that the healthcare system cannot move forward in terms of equity unless there is proper redistribution of opportunities through economic politics. Even though there have been tremendous advances in genomics or telemedicine, it basically serves wealthy classes. The state can thus become more of an aggressor of inequities by focusing on profit-driven adventures (Sen, 1999; Farmer, 2005).

Conversely, the case of Nigeria presents how inequities created by poverty, inadequate infrastructure, and bad governance produce inequities in global health. Nigeria spends less than 5% of its GDP on healthcare, making it below the WHO’s minimum threshold of service coverage (WHO, 2025). More than 70% of Nigeria’s health expenditure is derived from private consumption expenditure, meaning that less affluent individuals do not have adequate access to quality healthcare (Adewuyi & Auta, 2022). The country’s National Health Insurance Scheme (NHIS) provides less than 10% of the population (Okafor, 2024). Rural medical facilities lack drug supplies, sanitation facilities, and medical personnel. Nnamani & Erinosho (2023) described, malfeasance being perpetuated by inequity by channeling resources and misdirecting funds designed for fundamental services. Using the concept of structural violence defined by Farmer (2005), the inequities experienced by Nigeria shows structural violence perpetuated by negligence of diseases like Malaria, tuberculosis, or Maternal Healthcare inequities, which still pervade in the continent despite the knowledge of proper interventions existing in more advanced countries.

The COVID-19 pandemic has exposed this existing inequality. The structural divide in the US led to issues with vaccination distribution and death tolls. There was a near ten-fold difference in vaccination distribution among minority races, not from hesitancy but from exclusion from proper healthcare (CDC, 2023). Also, by the end of 2022, only a 20% of the population of Nigeria was completely vaccinated against the COVID-19 owing to issues of distribution, misinformation, as well as lack of proper vaccination storage facilities (Ogunniyi et al., 2023). The issue exposed exemplifies that existing global inequality transcends mere differences in medical capability. Mbembe (2020) introduces the concept of necropolitics, explaining how current global politics, both economically and structurally, create boundaries within which lives matter. The concept of countries with deeper pockets stockpiling vaccinations while countries in poverty waited in line aptly fits Mbembe’s explanation. WHO (2021), called this scenario a ‘vaccine apartheid’ owing to global patents.

The World Health Organization’s policy guidelines, established through Sustainable Development Goal 3 on "Good Health and Well-being," understand that achieving equal health

outcomes must consider various issues beyond the realms of healthcare itself, like gender equity, education, and adequate work (WHO, 2025). However, aftershocks of the global financial crisis" created by neoliberal policies tend to undermine economic agendas in favor of more market-oriented models (Kickbusch, 2014). As Kickbusch (2014) and Marmot (2015) discuss, the key to achieving equal outputs in healthcare is less dependent on healthcare itself and more dependent on income inequality, education, or community engagement. In both US and Nigeria, there is an understanding that while there is a focus on differing structural issues, there still seems to be a stringent focus on "health as a reflection of issues of injustice.

This requires, therefore, a transformation of the system in place rather than a series of specific program interventions. Farmer (2005) argues the need for a "preferential option for the poor," which requires policies deliberate intention of benefiting the poor rather than counting on trickledown effects. Redistribution policies via progressive taxation, pre-school education, or universal healthcare coverage have been found to effectively address inequities in middle-income countries (Marmot, 2020). On the other hand, an overhaul of existing global institutions like the WTO or World Bank could contribute to maintaining a more equitable economic framework which can help developing states in their pursuit of health sovereignty (Ho, 2021). Until then, advances in medicine will continue benefiting the few rather than the many.

Ultimately, the issue of global health inequities is an issue of the society prioritizing progress over parity. Both the US and Nigeria illustrate how disadvantages in terms of health can never be detached from disadvantages in terms of power. While both countries are rich in different ways, they both lack the provision of the universal right of all individuals to good health. The concepts of human development as an enhancement of capability by Sen (1999), the structural violence argument by Farmer (2005), and the focus on the role of the social determinant by the World Health Organization (2008) all provide fundamentally the same insight into global health inequities. The global gaps in health can never be ended by medicine alone, by transforming the very structures on which matters of lives hang.

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